ED side-room prioritisation

All UHB emergency departments · Who gets the side room first when rooms run out · Based on NIPCM (England) and Appendix 11 v1.1 (Mar 2026)

No negative-pressure rooms in our EDs
Airborne: single room, door shut, patient masked, staff FFP3.

Care before rooms
Never delay resuscitation or hide a deteriorating patient.

Isolate on suspicion
Don't wait for results. Mask anyone with respiratory symptoms.

1	**SUSPECTED HCID** STOP. Not covered by this sheet.	• **VHF, MERS, avian flu, novel pathogen** + travel or exposure • Nearest single room, door shut, minimal staff • Trust HCID pathway: **call on-call infection consultant and UKHSA HPT now**
2	**AIRBORNE** Single room, door shut, patient masked, staff FFP3. Call IPC.	• **Measles**: fever + rash; exposure, travel or non-immune • **Chickenpox**, disseminated or immunocompromised zoster, unexplained vesicular rash • **Probable pulmonary TB**: cough >2–3 wk, weight loss, night sweats, abnormal CXR, risk factors • **Possible MDR-TB**: escalate to micro/ID for transfer (needs negative pressure) • Mpox with respiratory symptoms or severe disease
3	**ENTERIC** Single room with en-suite or own commode. Soap and water, not gel.	• **Active vomiting ± diarrhoea** (norovirus-like): top of this priority • **C. difficile** suspected or confirmed • Other infective gastroenteritis
4	**HIGH-CONSEQUENCE DROPLET / CONTACT** Single room strongly preferred.	• **Meningococcal disease, invasive group A strep**: until established on antibiotics • **Pertussis, diphtheria** • **CPE** known or high risk (higher if incontinent or wounds) · Candida auris* · crusted scabies • Mpox without respiratory symptoms · PVL S. aureus
5	**RESPIRATORY VIRUSES** Room if free; otherwise cohort same confirmed virus, ≥1 m apart.	• **Influenza, COVID-19, RSV**: rapid test early, then cohort • **Move up** if AGP (NIV, high-flow), uncontrolled cough or can't mask • Other droplet infections (mumps, rubella, adenovirus, Mycoplasma etc.)
6	**LOWEST** Bay acceptable once rooms run out. Discuss with IPC.	• **MRSA, ESBL, VRE carriage**: continent, no wounds, no shedding • Localised covered shingles · extrapulmonary TB (pulmonary excluded) • Not infectious: known IBD flare, laxatives, psoriasis; chronic dry cough, no TB risk, normal CXR
P	**PROTECTIVE** Parallel to the ladder, not last.	• **Neutropenia** (<0.5 × 10^9/L), transplant, severe immunosuppression • Agree with senior and haemato-oncology. Never next to infectious patients.

No side room available? Bridge the gap

- **Mask the patient** (FRSM) if tolerated. Not always appropriate on oxygen, if distressed, or in some paediatric and mental health settings.
- **Out of the waiting room:** ≥1 m from others, best-ventilated, most remote space. Assess infectious patients first.
- **Airborne suspect:** masked, separate, well-ventilated space; tell IPC now.
- **Vomiting or diarrhoea:** own cubicle and commode, contact precautions; expect to close the area for cleaning afterwards.
- Dedicated equipment, hand hygiene, door closed wherever there is one.
- **Cohort** only ≥2 patients with the same confirmed infection, unless the Board-assured winter plan says otherwise.

A room is not free until it has been cleaned.

* Appendix 11 asks for a room for C. auris only in high-risk settings; see appendix. Full lists and detail: appendix (page 3 onwards).

Two patients both need the last room

1. **Clinical safety first.**
2. **Higher priority** is allocated the room.
3. **Same priority:** likelihood × consequence (MDR-TB before drug-sensitive TB).
4. **Can't be contained?** Incontinent, uncontrolled cough, AGP, can't mask.
5. **Can't be cohorted?** Confirmed flu can go to a flu bay.
6. **Result due soon?** Keep masked and apart until it returns.
7. **Two rooms, one en-suite?** En-suite to the enteric case.
8. **Plan for the other patient:** space, precautions, who is nearby.
9. **Free a room:** de-isolate anyone cleared.
10. **Document and escalate** to site manager and IPC. Incident report if a Priority 2 patient can't be isolated.

Both patients keep the precautions their infection needs.

How suspicious is enough?

Condition	Room if probability ≥
HCID	Any credible link
Measles	1–5%
Chickenpox / disseminated zoster	5–10%
Possible MDR-TB	~5%
Drug-sensitive pulmonary TB	10–20%
Vomiting / diarrhoea	~30%; always if vomiting or incontinent
C. difficile	20–30%
CPE	5–10%; known positive always
Meningococcal / invasive GAS	~20%
Flu / COVID-19 / RSV	~50%, or test and cohort

Expert opinion, not validated. Expect over-isolation for TB: in one ED study only 1 in 4 suspected cases had TB.

De-isolate and turn round

- Review every placement daily and when results return.
- Negative flu/COVID/RSV test: free the room.
- TB: IPC/TB team decides.
- Norovirus / C. difficile: usually 48 h symptom-free.
- Bacterial droplet infections: once established on antibiotics.
- After an AGP: leave empty for the time IPC sets.

Cleaning before reuse

- Bag waste and linen in the room.
- Decontaminate reusable equipment.
- 1,000 ppm chlorine, top to bottom, cleanest to dirtiest.
- Toilet, commode and touch points.
- Tell the nurse in charge when ready.

Notify and protect staff

- Notify the HPT on suspicion; don't wait for the lab.
- FFP3 must be fit-tested; no fit test = hood.
- Exposure in a shared area: tell IPC and HPT.

A1 · Status of this guidance

NIPCM and Appendix 11 give the room type for each infection but no national pathogen-by-pathogen priority order. The NHS England Isolation Prioritisation Tool ranks by risk, not by pathogen. The priorities here are a synthesis of three evidence reviews: consensus, not mandated. Trust IPC policy overrides this sheet; ask IPC or on-call microbiology if unsure.

A2 · Priorities in full

Priority	Who	Room, PPE and notes
1 · Suspected HCID	VHF, MERS, avian influenza, novel pathogen: compatible illness plus travel or exposure.	Nearest single room, door shut, fewest staff. Do not use this sheet or the NHS tool. Trust HCID pathway (Identify · Isolate · Inform): on-call infection consultant and UKHSA HPT. Clade I mpox is no longer an HCID (ACDP, Mar 2025).
2 · Airborne	• **Measles:** fever + rash, coryza or conjunctivitis; exposure, travel or non-immune • **Chickenpox** · disseminated or immunocompromised zoster · any unexplained vesicular rash until excluded • **Probable pulmonary or laryngeal TB:** cough >2–3 wk, haemoptysis, night sweats, weight loss, abnormal CXR, migrant/homeless/HIV • **Possible MDR-TB:** previous treatment, contact, high-burden country. Highest consequence. • **Mpox** with respiratory symptoms, severe disease or extensive lesions	• No negative-pressure rooms in UHB EDs: single room, door shut, patient masked (FRSM); call IPC. • MDR/XDR-TB needs a negative-pressure room: escalate at once to micro/ID about transfer. • Staff: fit-tested FFP3 or hood for measles, chickenpox, TB, respiratory mpox. Disseminated zoster: Appendix 11 lists FRSM, FFP3 for AGPs. • Isolate at a low level of suspicion: one missed case exposes the waiting room. • Already in a shared area: tell IPC and the HPT (contact tracing, PEP).
3 · Uncontained enteric shedding	• **Active vomiting ± diarrhoea** (norovirus-like): top of this priority. One episode in a bay can close the bay or department. • **C. difficile** suspected or confirmed: watery stool, Bristol 6–7, ≥3 in 24 h, recent antibiotics or hospital stay • **Other infective gastroenteritis:** Salmonella, STEC/O157, rotavirus, hepatitis A (single en-suite in App 11)	• Single room with en-suite or dedicated commode. • Isolate at symptom onset, not when the result returns. • Contact precautions. Soap and water for hands, not gel. • FRSM if vomiting; no other RPE. • Where possible keep en-suite rooms for enteric cases.
4 · High-consequence droplet / contact	• **Meningococcal disease** (or meningitis of unknown cause / septicaemia) and **invasive group A strep** • **Pertussis · diphtheria** (risk to infants and unvaccinated contacts) • **CPE** known or high risk. Top of this priority if incontinent, with wounds or uncontrolled secretions. • **Candida auris**‡ · **crusted scabies**† • **Mpox** without respiratory symptoms, severe disease or extensive lesions • **PVL S. aureus** (single en-suite)	• Single room strongly preferred; en-suite for contact organisms. • Bacterial infections: until established on appropriate antimicrobials. Reviews use ~24 h; App 11 gives no hour count, so follow local policy. • CPE: single en-suite in App 11. • C. auris ranks alongside CPE in the reviews, but App 11 asks for a room only in high-risk settings (ICU, oncology/haematology). • Droplet: FRSM within 1 m; FFP3/hood for AGPs. Contact (CPE, C. auris): gloves/apron per risk assessment, no RPE.
5 · Common respiratory viruses etc.	• **Influenza, COVID-19, RSV** • **Other droplet infections**‡ (single en-suite in App 11): mumps, rubella, adenovirus, parainfluenza, Mycoplasma, EV-D68, pneumococcal/Hib pneumonia (until on antimicrobials), parvovirus B19 (only before the rash)	• Single room if free; otherwise cohort by confirmed pathogen: ≥1 m apart, door or bay closed, FRSM on patients. • Rapid PCR/POCT early. Untested: a spare room while waiting, then move on the result. • Move up to a single room if AGP (NIV, high-flow O_2), uncontrolled cough, or cannot mask or comply (delirium, dementia, young child). • Never cohort unconfirmed or different infections, immunosuppressed patients, or patients unlikely to comply. Mixed cohorts only under a Board-assured preparedness plan.

6 · Lowest (bay acceptable)	• **MRSA, ESBL, VRE carriage**‡: continent, no wound, diarrhoea or respiratory shedding. (One review would add continent CPE carriers; App 11 says single en-suite: follow local IPC.) • Localised, covered shingles (immunocompetent) • Extrapulmonary TB with pulmonary disease excluded and no draining lesion • Pseudomonas, Acinetobacter, Serratia, Stenotrophomonas, Pneumocystis, B. cereus: room only in high-risk settings (ICU, oncology/haematology) per App 11	• Bay with standard ± contact precautions once rooms are exhausted. • This is rationing, not a claim that no room is needed. Discuss with IPC. • **Not a reason for a room:** diarrhoea or rash with a known non-infectious cause (IBD flare, laxatives, IBS, psoriasis); chronic dry cough with no TB risk features and a normal CXR (mask, main ED).
P · Protective (parallel)	Neutropenia (<0.5 × 10^9/L), post-transplant, severe immunosuppression.	Single room, door shut, apart from infectious patients. The room protects them from the department, so do not rank them last by default. Agree with the senior and haemato-oncology. Never bay with infectious patients or cohort with a different infection.

† reviews only, not in App 11 · ‡ placed by analogy (App 11 gives room type, no rank)

A3 · Triage screen

Fever + rash · vomiting/diarrhoea · cough + TB risk · travel · previous MDRO/CPE · hospital stay abroad. Mask anyone with respiratory symptoms. Isolate on suspicion, not on the result.

A4 · Two patients both need the last side room: full rules

Apply in order. Both patients keep the precautions their infection needs.

#	Rule	Detail
1	**Clinical safety first**	Resuscitation and visibility come before isolation. Isolated patients get less contact and more supportive-care failures.
2	**Match the room to the need**	Airborne suspects need a closed door away from the waiting area. Enteric cases gain most from an en-suite. Two rooms, one en-suite: the en-suite goes to the enteric case.
3	**Higher priority is allocated the room**	2 before 3, 3 before 4, and so on. Priority 1 (HCID) follows its own pathway.
4	**Same priority: likelihood × consequence**	Likely MDR-TB before probable drug-sensitive TB. Vomiting or incontinent norovirus before continent C. difficile.
5	**Who cannot be contained?**	Uncontrolled cough, incontinence, draining wound, AGP, cannot mask, cannot comply.
6	**Who can be cohorted?**	Allocate the room to the patient whose infection cannot be cohorted (unconfirmed, no matching bay). PCR-confirmed flu can go to the flu bay.
7	**Who leaves the room soonest?**	If one patient's rapid test is due within the hour, keep them masked and apart until the result.
8	**Plan for the patient remaining in the main department**	Oncology, transplant, pregnant, neonatal or unvaccinated children nearby lower the threshold to isolate. Plan their space, precautions and neighbours.
9	**Free a room**	Review current occupants: de-isolate anyone cleared, resolved, non-infectious or Priority 6; move patients waiting for beds to wards.
10	**Document and escalate**	Record the decision and review date in the notes and handover. Escalate to the site manager and IPC/microbiology. Incident report if a Priority 2 patient cannot be isolated.

A5 · Pre-test probability thresholds in full

Condition	Room once suspicion reaches about
Suspected HCID	Any credible link (HCID pathway)
Measles	1–5%: any compatible illness with exposure, travel or non-immunity
Chickenpox / disseminated zoster	5–10%
Possible MDR-TB	~5%
Drug-sensitive pulmonary TB	10–20%. Below about 1–5% (no risk factors, normal CXR) a room is not justified.
Active vomiting / diarrhoea	~30%, and always if vomiting or incontinent
C. difficile	20–30%
CPE	Known positive: always. Suspected: 5–10%
Meningococcal / invasive GAS	~20% (time-limited)
Influenza / COVID-19 / RSV	~50%, or test first and cohort

- **Basis:** two reviews that broadly agree. Expert opinion, not validated. Adjust for how many rooms are free; agree local changes with IPC.

- **Expect over-isolation for TB.** In one ED study only 25% of patients suspected of pulmonary TB had it, and a third of true cases were missed. About 3 in 4 isolated "TB suspects" being negative is the price of catching the rest.
- Use a structured TB assessment (risk factors, CXR, rapid NAAT/GeneXpert) so patients leave the room quickly when the answer is no.
- **Not adopted:** one review isolates only above 70% (10–70% wait in a cubicle for testing). This would leave suspected airborne infection in an open cubicle, and Appendix 11 specifies placement for suspected cases.

A6 · Review, de-isolation and cleaning

Situation	Action
All placements	Review at least daily and whenever a result returns, with IPC input. Write the review and de-isolation date in the notes.
Flu / COVID-19 / RSV	Negative rapid test frees the room within hours. Rapid PCR at the front door where possible.
TB	De-isolation is an IPC/TB-team decision, not an ED one.
Norovirus / C. difficile	Commonly 48 h symptom-free (local policy). Immunocompromised patients may shed for longer.
Bacterial droplet (meningococcus, GAS, pneumococcus, pertussis, Hib, diphtheria)	Until established on appropriate antimicrobials; the clinical team decides dose and duration.
After an AGP	Leave the room empty for the time IPC sets for that room's air changes (often at least 20 min; longer with poor ventilation). NIPCM minimum for ordinary single rooms: 6 air changes/h. Ask estates for true figures.
Cleaning before reuse	Bag waste and linen in the room. Decontaminate reusable equipment. Clean with 1,000 ppm available chlorine (or combined detergent-disinfectant), top to bottom, least to most contaminated. Extra attention to toilet, commode and touch points. Tell the nurse in charge when ready. **The room is not free until it is cleaned.**

A7 · Vital numbers and notification

- **1 m** minimum separation of infectious patients awaiting assessment or cohorted, door closed (NIPCM ch.2).
- **FFP3** (or hood) must be fit-tested and fit-checked each time; never worn by the patient. Staff without a fit test: hood.
- **AGPs** on a patient with suspected or confirmed droplet/airborne infection: FFP3 or hood, and a room if possible.
- **Notify on suspicion:** measles, TB, meningococcal, mumps, rubella, pertussis, diphtheria, mpox, hepatitis A, invasive GAS and others marked "Yes" in App 11. Phone the HPT within 24 h if urgent; written notification within 3 days. Don't wait for the lab.

A8 · NHS England Isolation Prioritisation Tool (v1.0, Oct 2024)

- Scores risk = probability × impact for one pathogen in one patient population. It does not rank pathogens and does not decide.
- "Isolation recommended? Yes/No" is a manual entry, then free text for reasoning, review date and de-isolation date.
- Meant for a multidisciplinary team (IPC, medical, nursing, operations, facilities). **Never for a suspected HCID.**
- Use it when two patients at the same priority both need a room, when the organism isn't on page 1, or to document a decision.

Probability (stop at the first "No")	Impact (up to 4 questions)
1. Population at risk of further exposure? No = very low	Severe disease after exposure?
2. Susceptible? No = low	Significant number of the wider population susceptible or at risk?
3. Highly transmissible, environmentally persistent, widely spread or antimicrobial-resistant? No = moderate	Negative effect on current service and future capacity?
4. Transmission likely? No = high; Yes = very high	Effective treatment and controls in place or possible?

Probability ↓ / Impact →	Very low	Low	Moderate	High	Very high
Very low	Very low	Low	Low	Moderate	Moderate
Low	Low	Low	Moderate	Moderate	Moderate
Moderate	Low	Moderate	Moderate	High	High
High	Moderate	Moderate	High	High	Very high
Very high	Moderate	High	High	Very high	Very high

Highest overall risk is allocated the room first. Assessment must be ongoing and include de-isolation.

A9 · Where the sources disagree

Issue	Disagreement	Position taken here
National rank	App 11 gives room type per organism (mostly "single en-suite room"); no numeric rank.	Priorities reconciled from three reviews. All three put HCID and airborne first.
HCID	One review lists HCIDs in its top tier; the NHS tool must not be used for them.	Priority 1 with a stop instruction: cannot be overlooked, not ranked against other infections.

Norovirus	One review ranks it with TB and measles; the others just below.	Heads Priority 3: needs a different kind of room from airborne cases.
Continent CPE carriers	One review accepts a bay under scarcity; App 11 says single en-suite.	Not adopted. All three accept MRSA/VRE/ESBL carriage in a bay, which departs from App 11 for MRSA. Check local policy.
TB threshold	Two reviews 10–20%; one 70%.	10–20% (see A5).
De-isolation timing	One review uses ">48 h of antibiotics" for pneumococcal disease.	App 11 wording: "established on appropriate antimicrobial treatment"; clinical team decides.
Room engineering	One review quotes US CDC values (12 air changes/h, −2.5 Pa).	Use NIPCM and Trust estates figures.
Scottish v English App 11	One review cites Scottish App 11 (v4.0, R1/R2/R3). English App 11 (v1.1) uses "negative pressure isolation room/suite" v "single en-suite room".	English App 11 v1.1 used.
Neutropenia	Ranked last by one review, "conditional" by another.	Parallel priority: NIPCM ch.2 prioritises patients at increased risk of adverse outcomes.

A10 · Quality of the evidence

- **Strong:** airborne organisms spread across shared airspace; masking, separating and rapid testing help.
- **Weak:** that single rooms themselves reduce transmission (low or very low certainty for every organism; no randomised trials; mostly bundled outbreak measures). Isolation has real harms: anxiety, depression, less clinician contact, possibly more adverse events.
- **Expert opinion only:** every numeric threshold and the order within priorities.
- The reviews' epidemiology figures disagree in places (e.g. TB notifications) and two cite non-authoritative web sources (e.g. a vendor blog, Substack), so no incidence figures are used on this sheet.

A11 · Sources reviewed

NHS England NIPCM home page (v3, updated 28 July 2026) · Chapter 1 SICPs, §1.1 patient placement · Chapter 2 TBPs, §2.1–2.5 · Appendix 11: optimal patient placement and RPE, v1.1 (March 2026) · NHS England HCAI compendium (v22, July 2026) · NHS England National IPC Isolation Prioritisation Tool (v1.0, Oct 2024) · Prioritising Scarce Side Rooms for Suspected or Confirmed Infection in the UK ED: Evidence Review (Sept 2026) · Prioritising Single-Room Isolation: A Rapid Literature Review with GRADE-style Certainty Assessment (30 Sept 2026) · Evidence-Based Prioritisation of Single-Room Isolation in UK EDs (undated). The three reviews are not official guidance.

Not a substitute for clinical judgement, Trust IPC policy, or advice from IPC, microbiology or the UKHSA health protection team.